



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : وفاء عبد المنعم امين عبد المولى

العنوان : ///

الحى / المدينة : عين شمس

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 03-10-2026

رقم البطاقة : 27310260103823 رقم التليفون : 01118546763

العمر (تاريخ الميلاد) : 26-10-1973 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/ د/ د/ د/

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: وفاء عبد المنعم امين عبد المولى National ID: 27310260103823	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

وفاء
عبد المنعم امين عبد المولى
٢٨ مقصود الصفاوي - الملك فيصل - الجيزة
بولاق النور - الجيزة

٢٧٣١٠٢٦٠١٠٣٨٢٣
KN2346822

DMS
فيلار

٢٠٢٤/١٠/٢٣
٢٠٢٣/١٠/٢٣
٢٠٢٣/١٠/٢٣
٢٠٢٣/١٠/٢٣

حصوله على بطول تجارة
انقضى
صلاح الدين عطية حسن سميح
الطاقة سارية حتى ٢٠٢٣/١٠/٢٣

مترجمة
مسلمة
انقضى
صلاح الدين عطية حسن سميح
الطاقة سارية حتى ٢٠٢٣/١٠/٢٣

13/11/2024, 10:07

Code 42 - Name: A MDT form Harat - Name: E MDT form Harat



Ministry of Health and Population
Ministry of Health and Population

وزارة الصحة والسكان
Ministry of Health and Population



A-Patient ID:

1- Name:

ولاء عبد الله بن عبد الله

Sex:

MDT form
3- Medical number:
Female

B320/244

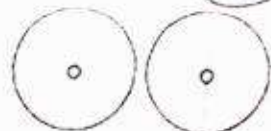
Old Med:

15/10/2024

2-ID number:

Patient Age: 51 Y

MDT



Post-NAC
PR

Family history:

Family history

Phone:

01118546763

BMI:

BMI

Marital status:

Marital status

NRS Score:

NRS Score

GROUP:

GROUP

Co-morbidity:

Co-morbidity

Menopausal status:

Menopausal status

Initial STAGE:

Initial STAGE

REASON OF MDT:

REASON OF MDT

Date of MDT:

13/11/2024

Breast Imaging:

in UOQ mass 24 x 25 mm 10 mm away from skin, satellite nodule from
1-5 o'clock nipple another mass 2.8 x 18 mm 17 x 8 mm at 4-5 cm
in pathological LN

Clinical data

Mets Workup:

ct cap: D2 L3 sclerotic
bone scan: free

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	4/8	2	Amplified	
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT:	

T	N	M
4b	2	0

Sonomammography

PATIENT HISTORY

pt received 13 wks taxol + 4 cycle phesgo last 27/10/2024

stop ch in view of v. poor tolerance

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR LT MRM (meanwhile resume phesgo after new echo)

SIGNATURE

MEDICAL ONCOLOGY	RADIOLOGY
SURGERY	PATHOLOGY
Psychiatric	Nutrition

Patient Name : الأستاذة / وفاء عبد المنعم امين عبد المولى	Request No. : 13001984
Patient No. : 27310260103823	Request Date : 02/12/2024
Path No. : 4717/24	Verification Date : 14/12/2024
Sex : Female	Financial Category : أورام ناز السلام - نفقة التوبة
Age : 51 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Left MRM measured 27x26x7 cm with covering skin ellipse measured 26x25 cm.
- * Tumor extent: An ill defined greyish white rubbery to firm tumor bed site.
- * Size: 2x1 x1 cm.
- * Skin involvement: skin showed edema.
- * Distance from margins: 3.5 cm away from deep margin, and 3 cm from skin.
- * Additional findings: A firm greyish white nodule measured 3x1 cm (UOQ).
- * Rest of the breast showed multiple variable sized scattered greyish white rubbery foci measured up to 1.5x1 cm with least deep margin 2.5 cm and 2 cm from skin.
- * Lymph nodes number and size: 17 LNs up to 1.7x1 cm.
- 1&2) TBS. 3) Deep margin. 4 to7) Scattered rubbery grey white areas. 8,9) Firm nodule. 10) N&A. 11,12 & 13) LNs.

Grossing by Dr. Yasmine Esam.

Microscopic Examination:

- * Histological type: Multicentric residual Invasive duct carcinoma with focal mucinous activity.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Maximal tumor diameter: 3 cm.
- Actual tumor cellularity: 40%, tumor necrosis index: 60%
- * Ductal carcinoma in situ: Focal DCIS (6%).
- * Lymphovascular emboli: Positive.
- * Perineural invasion: Not identified.
- * Skin involvement: Free nipple & areola.
- * Lymph node involvement: positive in (3/18) with focal extracapsular exention, largest

Verified by :

Printing Date : 14/12/2024 12:00:24

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt

الأستاذ / وفاء عبدالمنعم امين عبدالمولى :
:27310260103823
:4717/24
: Female
: 51 Y

Referring Doc :

Request No. : 13001984
Request Date : 02/12/2024
Verification Date : 14/12/2024
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

metastatic deposit (13 mm).

- * Resection Margins: Free.
- * Histologic response to therapy: Partial.
- * Microcalcifications: Not identified.

Diagnosis:

Left breast cancer, post NACT, MRM specimen:

Residual invasive duct carcinoma with focal mucinous activity.

- Maximal tumor diameter: 3 cm.
- Actual tumor cellularity: 40%, tumor necrosis index: 60%.
- Free Resection Margin.
- Free nipple & areola.
- Lymph nodes: (3/18) with focal extracapsular extension.
- Partial pathological response to therapy.
- Stage: ypT2 ypN1 ypMx.
- RCB: 3.709, RCB class III.

Signature

Prof. Dr. Lubna Omar.
Dr. Rufaida Abdullah.

Verified by :

Printing Date: 14/12/2024 12:00:24

Page 2 of 2

Printing By : Mrs. Mayada Hegazy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mizr El Qadema - Cairo, Egypt

PATIENT NAME: WAFAA ABD AL MONEM AMEEN ABDELMOLA

GENDER: Female

DATE : 23-Apr-2024 11:58 AM

PATIENT ID: 10124240829

AGE: 50 y

ORDER: 2811422,2811423

Multislice post contrast CT scan of the chest, abdomen and pelvis revealed:**(i) Chest:**

- Most of right breast parenchyma out of field of exam.
- Left axillary level I, II & III pathological lymph nodes and amalgamated the largest one measures 6.2 x 6.6 cm.
- No pulmonary nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Right thyroid nodule measures 15 x 20 mm for US correlation. For US correlation.

(ii) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density and enhancement with no focal lesions.
- Patent and homogeneously enhancing portal vein.
- Calcular G.B.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys apart from small cortical cysts, aorta and IVC.
- Underfilling of the urinary bladder showing no stones, masses or diverticulae.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- D2 and L3 tiny sclerotic foci for bone scan correlation

OPINION:**A known case of left breast cancer showing:**

- Left axillary pathologically enlarged lymph nodes.
- D2 and L3 tiny calcific focus for bone scan correlation.
- No pulmonary or hepatic metastatic deposits.

PATIENT: WAFAA ABD AL MONEM AMEEN ABDELMOLA 10124240829, Female, 50 y

Report version: 2, 24-Apr-2024 03:39 PM

Page 1 of 2



Patient Name: Wafaa Abd Al Monem Ameron Abd Al Mawla	Date: 04/08/2024
Exam: ultrasound ABDOMEN & PELVIS	Age: 50 Year(s)
Medical ID: 1530/2024	Pat. ID: 77123

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Mildly enlarged in size (about 17 cm at the right MCL), smooth outline, bright echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent.

Gall Bladder: -

- Average size, normal wall thickness, multiple variable sized hyperechoic mobile stones are noted; measuring 5 - 15 mm.
- CBD is not dilated measuring 5 mm.

Spleen: mildly enlarged size (About 13.5 cm), displaying homogenous echopattern.

Kidneys: -

- Are of normal size and site, normal echogenicity, no stone or backpressure changes, no solid or cystic lesion.
- Both ureters are noted dilated.

URINARY BLADDER: Normal filling, Normal wall thickness, no stones or diverticulae or solid lesion detected.

Uterus: -

- Anteverted, average sized uterus. Endometrial thickness measures about. mm, no focal lesions.

Ovaries: - both are average in size with no dominant follicle could be detected.

- No free fluid in the Douglas pouch.
- Para-aortic and pancreatic area: no gross lesion

Opinion:

- Bright hepatomegaly likely fatty for liver profile correlation.
- Mild splenomegaly.
- Calcular gall bladder.

Best Regards,
Dr. Mostafaz Adel, MSc.

PATIENT NAME: WAFAA ABD AL MONEM AMEEN ABDELMOLA
GENDER: Female
DATE : 08-Apr-2024 01:20 PM

PATIENT ID: 10124240829
AGE: 50 y
ORDER: 2791974,2792001

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- left breast palpable lump .

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities of both breasts . (ACR b)
- left UOQ and LOQ irregular shaped spiculated margin high density mass lesions .
- This was confirmed by Complementary Ultrasound which shows left irregular shaped hypoechoic mass lesion measuring 24x25mm , 10mm away from overlying skin , with mild internal vascularity by color doppler study seen at 1-2 O'clock , surrounded by similar irregular hypoechoic satellite nodules the largest measuring measuring 11x10mm occupying area from 1 to 5 oclock with suspected extension to nipple??
- another similar lesions seen at 4-5 O'clock measuring 28x18mm and 17x8mm
- another indeterminate circumscribed hypoechoic nodule seen at 12 oclock measuring 10x5mm
- no right solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- left enlarged pathological axillary (level I,II,III) more than three with

PATIENT: WAFAA ABD AL MONEM AMEEN ABDELMOLA, 10124240829, Female, 50 y

Report version 2, 08-Apr-2024 02:13 PM

Page 1 of 3



the largest amalgamated nodes measuring 5x2,7cm with muffled hilum.

- no supraclavicular pathological lymph nodes .
- Right nonspecific axillary lymphadenopathy.

OPINION:

- left breast UOQ and LOQ malignant looking mass lesions as above described with ipsilateral pathological (level I,II,III) axillary lymph node for histopathological correlation (BIRADS 4c) .
- No suspicious finding of right breast (BIRADS2).

Referral to clinic

Reported By,

Dr. Amara Abdelsalam, FEBR, MSc

08-Apr-2024 02:13 PM

PATIENT: WAFAA ABD AL MONEM AMEEN ABDELMOLA, 10124240829, Female, 50 y

Page 2 of 3

Report version 2, 08-Apr-2024 02:13 PM



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Patient Name: Wafaa Abd Al Moneim Ameen Abd Al Mawla	Date : 03/10/2024
Exam : C.T CHEST	Age : 51 Year(s)
Medical ID:	Pat. ID: 77123

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST & PARA NASAL SINUSES:-

1. PNS:

- Right maxillary moderate mucosal thickening.
- Clear scanned paranasal sinuses with intact bony boundaries.
- *No CT evidence of hyper-densities within the mucosal thickening or bone destruction/erosions.*
- No air/fluid levels are seen.
- Patent both osteomeatal units and sphenoethmoidal recesses.
- The nasal septum is deviated the left side with bony spur formation.
- Clear nasopharynx.

2. CHEST:

- Clear both lung fields
- No evidence of pleural effusion, pleural thickening or masses seen.
- No pathological mediastinal lymph nodes.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Well-defined right thyroid lobe nodule measuring 1.9x1.6 cm.
- Upper abdominal cuts show multiple gallbladder stones ,

OPINION:

- *No CT evidence suggestive of fungal sinusitis.*
- *No CT evidence of pneumonia.*

Dr. Mohamed Mohsen, M.D.

PATIENT NAME: WAFAA ABD AL MONEM AMEEN ABDELMOLA

GENDER: Female

DATE : 25-May-2024 01:35 PM

PATIENT ID: 10124240829

AGE: 50 y

ORDER: 2860540

99MTC-MDP BONE SCAN**Desired Information: -**

Skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.

Regenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

26-May-2024 01:44 PM

PATIENT: WAFAA ABD AL MONEM AMEEN ABDELMOLA, 10124240829, Female, 50 y

Report version: 1, 26-May-2024 01:44 PM

Page 1 of 1



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والتي تقدم الخدمات الطبية